

Public Health Emergency Operations Center
"COUSP DRC"

MVE-17 Incident Management System

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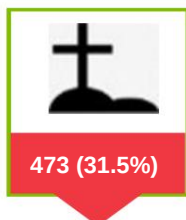
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°049/MVB_02/07/2026

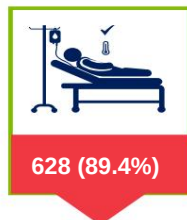
Provinces affected (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (36)	- Ituri (24/36): Aru, Aungba, Bambu, Bunia, Damas, Drodro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima and Lolwa. - North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. - South Kivu (1/34): Miti-Murhesa
Reporting date	July 2, 2026
Publication date	July 3, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed cases
and case fatality rate



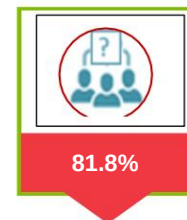
Patients in
isolation -
hospitalization



Cumulative number
of recoveries



Suspected cases of
the day



Contact tracing rates
for the 2 provinces

* Data updated following the DHIS2 database cleaning and harmonization process, including the consolidation of information from health zones, laboratories and case management structures.

Suspicious deaths occurring in the community are subject to investigation.

further investigation with a view to possible reclassification (Non-cases, probable cases)

1. HIGHLIGHTS

- **Official visit of the South African President to Kinshasa** to support the fight against the Ebola Bundibugyo virus disease epidemic and to strengthen cooperation between the Democratic Republic of Congo and South Africa.
- **Forty-two (42) new confirmed cases, including 7 community deaths**, were reported on July 2, 2026, in the provinces of Ituri (**33 cases, including 5 deaths**) and North Kivu (9 cases, **including 2 deaths**). These confirmed cases originate from the health zones of **Rwampara (14), Bunia (10), Mongbwalu (3), Bambu (2), Mandima (2), Lita (1), and Tchomia (1)** in Ituri, and **Butembo (4), Katwa (2), Beni (2), and Kyondo (1)** in North Kivu.
- **Fourteen (14)** deaths were recorded among patients with Ebola virus disease in Ebola treatment centers in the provinces of Ituri (6 at the Mongbwalu Ebola Treatment Center, 3 at the Bunia Ebola Treatment Center, 2 at the Rwampara Ebola Treatment Center, 1 at the ISTM Ebola Treatment Center and 1 at the CME Ebola Treatment Center) and North Kivu (1 at the Katwa Ebola Treatment Center).
- **Sixteen (16)** patients with Ebola virus disease, including 15 in Ituri province (6 at CTE Mongbwalu, 3 at CTE CME, 3 at CTE Bunia, 2 at CT Bambu and 1 at CTE Rwampara) and 1 at CTE Katwa in North Kivu province, have been declared cured after obtaining negative control tests.
- Installation of diagnostic laboratories in the health zones of Komanda, Nizi, Mambasa and Nia-Nia, in the province of Ituri.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For over two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola virus disease (EVD) outbreak, which was then raging in North Kivu. The current epidemic, caused by the Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total estimated population of nearly 15 million, with massive internal displacement and cross-border flows to neighboring countries.

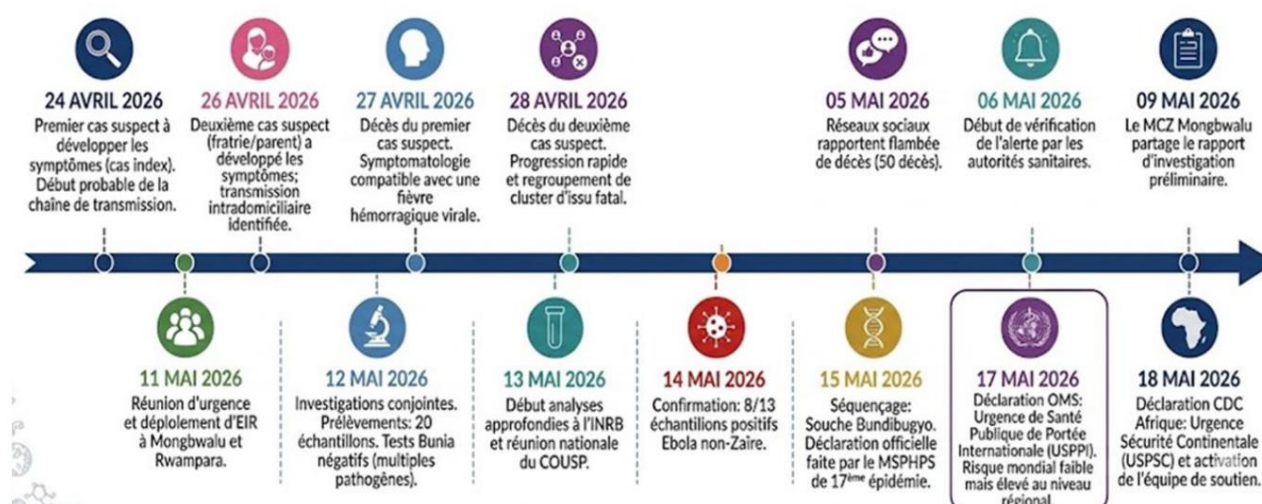


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of July 2, 2026

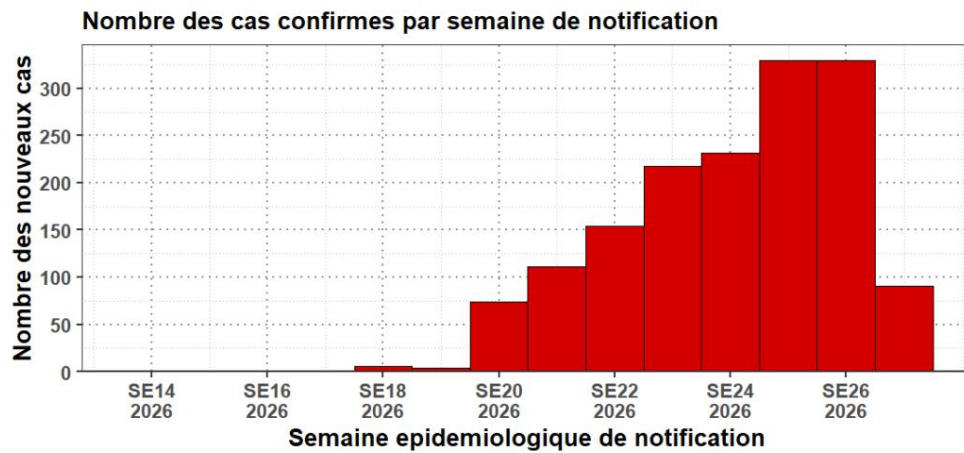
Table 1. Distribution of confirmed cases and deaths by affected province as of July 2, 2026.

Province	Confirmed cases	Deaths	Lethality	Affected health zones	New confirmed cases (24 hours)
		(confirmed)			
Ituri	1,366	397*	29.1% 24 out of 36 (66.7%)	56.4% 11	33
North Kivu	133	75*	out of 34 (32.3%)	33.3% 1 out of 34	9
South Kivu	3	1	(2.9%)		0
Total	1,502	473	31.5%	36 out of 104 (34.6%)	42

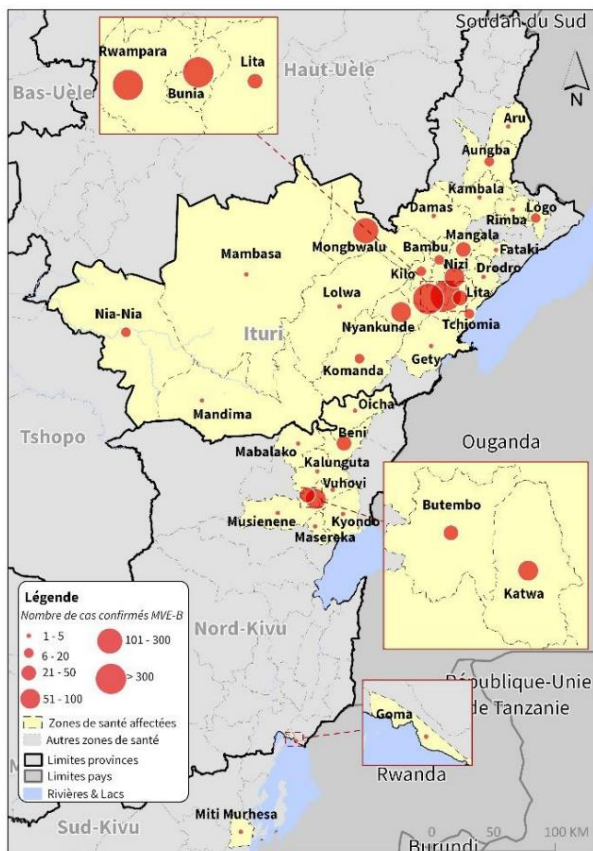
*13 deaths in Ituri and 1 death in North Kivu were recorded among patients with EVD (previous confirmed cases) in ETD/CT.

At the end of the day on July 2, 2026, no new health zones were affected. The number of affected health zones remains at 36/104 (section 3.1). Since May 26, 2026, South Kivu province has not recorded any confirmed cases, suggesting an absence of recent transmission.

3.3 Time-Based Analysis: An increasing number of confirmed cases is observed from one week to the next, reflecting ongoing community transmission of the disease, although the number of cases appeared to be stable during epidemiological weeks 23 and 24. The intensification of public health actions in epidemiological and biological surveillance (decentralization of diagnostic capabilities) contributed to early detection, confirming continued and increased community transmission by week 25. Compared to all other weeks in which cases have been recorded since the beginning of the epidemic, weeks 25 and 26 represent up to now the peak, i.e. more than 300 cases per week.



3.4 Epidemic mapping



Figures 2 and 3. Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of July 2, 2026.

Table 2. Distribution of confirmed cases and deaths of Ebola virus disease by province and health zone (July 2, 2026)

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Case fatality rate (CFR%)
ITURI	1366	397	29.1
Bunia	426	106	24.9
Rwampara	322	67	20.8
Mongbwalu	273	136	49.8
Nyankunde	95	16	16.8
Nizi	65	19	29.2
Lita	33	12	36.4
Komanda	19	5	26.3
Bamboo	18	5	27.8
Kilo			11.1
Mangala	9 24	1 12	50.0
Tchomia	15	2	13.3
Damascus	5	0	0.0
Aungba	6	2	33.3
Rimba	3	0	0.0
Aru	3		33.3
Mambasa	2		50.0
Logo	7	1	0.0
Drodro	3	1	100.0
Gety	1	0	0.0
Kambala	2	3	100.0
Nia-Nia	12	0	41.7
Fataki		2	0.0
Mandima	1 4	5 0 1	25.0
Lolwa			100.0
Other areas not yet identified	1 17	10	0.0
NORTH KIVU	133	75	56.4
Butembo	38	16	44.1
Katwa	51	35	64.2
Blessed	28	16	60.7
Oicha	3	2	60.0
Kalunguta	2	1	50.0
Kyondo	3	2	66.7
Goma	2	0	0.0
Masereka		0	0.0
Vuhovi			100.0
Mabalako		1	0.0
Musienene	1	0	66.7
SOUTH KIVU	1 1 3 3	2 1	33.3
Miti-Murhesa	3	1	33.3
TOTAL	1,502	473	31.5

In the past 24 hours, **42 new confirmed cases and 21 deaths have been recorded** in the provinces of Ituri (33 cases and 18 deaths) and North Kivu (9 cases and 3 deaths), highlighting the progression of the epidemiological situation of the 17th Ebola Virus Disease (EVD) epidemic in the Democratic Republic of Congo. Thus, the cumulative total stands at 1,502 confirmed cases and 473 deaths, expressing an overall case fatality rate of 31.6%.

Provincial dynamics and case concentration

The **Ituri** province alone accounts for **90.9%** of cases (n=1,366) and **83.9%** of deaths (n=398), representing a case fatality rate of **29.1%**, making it the absolute epicenter of the epidemic. The health zones of Bunia (426 cases), Rwampara (322 cases) and Mongbwalu (273 cases) remain the most affected, accounting for approximately 67.9% of reported cases at the provincial level and 74.7% at the national level. Within these same health zones, moderate case fatality rates were observed in Rwampara (20.8%) and Bunia (25.1%), while Mongbwalu **also** experienced a high case fatality rate of **49.8%** (136 deaths out of 273 cases), suggesting persistent challenges in early intervention and access to care. It should be noted that 17 additional cases (with no associated deaths) have been reported, while the health zones of origin remain to be identified, which could indicate a geographical expansion requiring further investigation.

Severity of the epidemic in North Kivu

A total of 133 confirmed cases and 75 deaths have been reported in North Kivu province since the start of the epidemic, representing a **case fatality rate of 56.4%**. This province presents the most concerning profile in terms of severity, with a case fatality rate significantly higher than the national average. This high case fatality rate is concentrated in the health zones of **Katwa** (68.6%), **Oicha** (66.7%), **Kyondo** (66.7%), and **Beni** (57.1%). The health zone of Katwa leads in notification in terms of the number of cases (n=53), followed by the Butembo health zone (n=38) with a case fatality rate of 42.1%. A case fatality rate of 100% was exceptionally observed in the Vuhovi health zone

(with a small number of employees).

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri & Kinshasa

- Holding of the coordination meeting chaired by the Director General of INSP with the response team and technical and financial partners on field activities.
- **Official visit of the South African President to Kinshasa** to support **the fight against the Ebola Bundibugyo virus disease epidemic** and the strengthening of cooperation between the Democratic Republic of Congo and South Africa.

4.2 Epidemiological surveillance

Table 3. Management of epidemiological alerts (24h) as of July 2, 2026

Indicator	Ituri	North Kivu	South Kivu	Total
Report (D-1)				108
New Alerts		0	0	970
Total alerts for the day		608	13	1078
Alerts investigated		608		767
Investigation rate		608	13 9	71.2%
Alerts confirmed	Alive			150
	Deceased	108,349,457,150 31.1%	108,409 43 18 69.2%	63
Total suspected cases for today	150	61	2	213

- At the end of the day on July 2, 2026, **1078 alerts** were reported across the three provinces, **759 (70.4%)** were investigated, **213 suspected cases including 63 deaths were identified** (Table 3).

Table 4. Contact tracing of confirmed cases as of July 2, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri	9046	7,433	82.2%
North Kivu	2,314	1,858	80.2%
Total	11,360	9,291	81.8%

- Continued in-depth investigations and exhaustive listing and systematic follow-up of contacts around confirmed cases in the ZS (narratives of confirmed cases, number of contacts listed and followed per case).
- 341 new contacts** were listed, including **236** in 4 health zones in Ituri province and **105** in 3 health zones in North Kivu province.
- 11** contacts have **become suspects**, including **3** in Ituri province and **8** in North Kivu province.
- 269 contacts successfully completed their 21-day follow-up**, including **255** in 5 health zones in Ituri province and **14** in 1 ZS of the province of North Kivu.

Update on checkpoint and point of entry (PoC/PoE) activities

- Ituri** : Five (5) alerts were reported to the Points of Control (PoCs), including three cases of live individuals at the Mabakese PoC. Two cases were confirmed, one of which was referred to the Mambasa General Referral Hospital (HGR). Another case occurred before the ambulance arrived and after threatening healthcare workers. One case at the Mudzibalail PoC presented with agitation, sweating, and urinary retention. This case was reported, confirmed, and referred to the Medical Emergency Center (CME).
- North Kivu**: Two (2) live alerts were notified to the Mukulia and Makeke PoCs; one alert was validated and referred to the CT of the Beni General Referral Hospital; Refusals of screening remain significant at certain points, notably Pluto, Anzida, Rubingo, and Chamugamba. Reported reasons include denial of the disease, misinformation, low adherence to the response, and insufficient risk information.

Table 5. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC), as of July 2nd 2026

INDICATORS	Ituri	North Kivu	Overall
Proportion of PoC enabled and PoE enhanced	98.1% (54/55)	100.0% (14/14)	98.5% (68/69)
Number of people who went through PoE/PoC testing,	89,504	29,741	119,245
% of travelers screened, % of	96.7%	95.1%	95.9%
travelers who washed their hands, % of	96.3%	95.1%	95.7%
travelers made aware of the issue	99.4%	95.1%	97.3%
Number of alerts notified	5	2	7
Number of validated live alerts	3		4
Number of lifeless bodies intercepted today	2	10	2
Number of problematic contacts intercepted today; %			0
of validated live alerts referred to CT/CTE; % of	0	0	58.3%
deceased bodies swabed	66.6%	50.0%	0.0%
Number of positive results of referred suspected cases	100.0% 0	0.0% 0	0

4.3. Laboratory

- **Ituri** : 100 samples collected and analyzed (**positivity 33%; n=33**).
- **North Kivu** : 94 samples collected and analyzed (**positivity 9.6%; n=9**).
- **South Kivu** : 2 samples collected and analyzed, all came back negative.
- Installation of diagnostic laboratories in the health zones of Komanda, Nizi, Mambasa and Nia-Nia, in the province of Ituri.

4.4. Infection Prevention and Control (IPC)

Ituri: 21/27 households were identified and decontaminated around confirmed cases; 55 means of transport (ambulances, hearses) were decontaminated and 33 service providers were briefed on Ebola Virus Disease (EVD); 3 health establishments (HEs) were identified and decontaminated around confirmed cases in the health zones of Bunia, Rwampara, Mongbwalu, and Komanda; 9 public and specific places were decontaminated: office 1; 26/29 health assessments (HAS) were carried out with 2 postponements; IPC/WASH assessments were conducted on 9 HEs in the health zones of Mongbwalu, Bunia, Lita, Nyankunde, and Rwampara; 29 HEs were monitored and supported: Salama Clinic 87.1%; St. Raphael Medical Center 17.4%; St. Pierre Medical Center 58%; Umoja Health Center 35.5%; Bon Berger Clinic 50.2%. PS Rapha 50%, PS Guerison 39%, PS Maman Mwilu 40%; CM Ave Maria 30%, CS Poli de Frontières 41.9%; CS Nyakero 40%, CS Lengabo 68.2%; PS Tuluza 25.3%; CM Bethel 22.6%, PS La Sagesse 8.1%, PS Benjamine 14.5%, CM Karibuni 35.5%, CM Spaec Atlas 35.5%; CS Lengabo 68.2%; CS Bigo 59.6%; CM Sauvetage 25.3%, CM Gracia Katsina. 9,729/39 households have been identified and contain around confirmed cases; Decontamination of 10 means of transport (ambulance, hearses) in Nizi, Komanda, Mongbwalu and briefing during the formative supervision of 48 providers on General information on Ebola and Standard precautions; 6 ESS identified and decontaminated around confirmed cases in the ZS Nizi (isolation HGR PS Heritage), ZS Lita (HGR, CS Mutumbi and Bahwere), Nyankunde; Follow-up and support of 21 ESSs accompanied: CM Céleste, PS Umoja, CS Rwambuzi, CS Kindia, PS Change l'image, PS Tumaini, PS Tulizeni, PS SAMELO, Clin Grace de Dieu, CS Mudzi maria, CS bahwere SOTA, CS , HGR NYAKUNDE, PS THANK GOD, CS KOMBOKABO, CS SEDZABO, CS BIRINYAMA, CS IRUMU; 5 decontamination of IPC/EDS/Lab kits/supplies in the NIZI Health Zone (materials and IPC kits), Lengabo Health Center (Rwampara), Nyakunde Health Zone (9 mattresses: Emergency and Marabo Health Centers), Mongbwalu Health Zone (Faraje Health Center) of public and specific locations: CARE NGO office 1; 2 distribution of hygiene kits to households with confirmed cases in the Mongbwalu Health Zone; IPC/WASH assessment of 9 health centers in the Mongbwalu, Bunia, Lita, Nyankunde, and Rwampara Health Zones; Monitoring and support of 29 health centers; IPC/WASH assessment of 9: Celeste Community Health Center 24.2%; Rwambuzi Health Center 57.1%; Kindia Health Center 55.5%; Tumaini Health Center 43.5%; Tulizeni Health Center 58%; AMELO Health Center 9.7%; Clinical Grace de Dieu 37.1%, CS Bahwere 33%, Disp St Benoit: 19.4%; briefing of 52 community leaders and community representatives among prisoners (PCB); people who were briefed in households during the activity in Nizi and Bunia; ZS NIZI (materials and PCI kit), CS Lengabo (Rwampara), ZS Nyakunde (9 mattresses: PS S Households not found in the rings and 2/27 kits provided to households of decontaminated cases

4.5 Holistic care

- **Fourteen (14)** deaths were recorded among patients with Ebola virus disease in Ebola treatment centers in the provinces of Ituri (6 at the Ebola Treatment Center in Mongbwalu, 4 at the Ebola Treatment Center in Bunia, 2 at the Ebola Treatment Center in Rwampara, 1 at the Ebola Treatment Center in ISTM and 1 at the Ebola Treatment Center in CME and North Kivu (1 at the Ebola Treatment Center in Katwa).
- **Sixteen (16)** patients with Ebola virus disease, including 15 in Ituri province (6 at CTE Mongbwalu, 3 at CTE CME, 3 at CTE Bunia, 2 at CT Bambu and 1 at CTE Rwampara) and 1 at CTE Katwa in North Kivu province, have been declared cured after obtaining negative control tests.

- At the end of the day on July 2, 758 hot meals were served, including 135 to confirmed cases, 175 to suspected cases, 420 to PPLs and 28 to accompanying persons.

North Kivu:

- A 3-month-old child confirmed at the Beni General Referral Hospital; Monitoring of nutritional care activities at the Beni, Malepe and Bundji General Referral Hospitals; Data collection and preparation of daily reports.
- Distribution of hot meals to 7 suspects in Malepe (MSF H), 11 at the HGR of Beni including 3 confirmed and 8 suspects (IMC), 2 suspects in Bundji (IMC), 30 frontline providers at the HGR of Beni (IMC);
A 21-day-old infant on SLM
- 6 hot meals distributed to accompanying persons (3 HGR, 2 Bundji (IMC) and 1 in Malepe (MSF H)).3 caregivers made aware of ANJU.

Table 6. Patient movement within healthcare facilities (CTE/CT/CI) as of July 2, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	487	135	22	644
Total admissions (24h)	51	21	0	72
Exits (24h)				
Deceased (Suspects/Conf)		1	0	16
No case	15:45	7	3	55
Cured	15		0	16
Escapees (Suspects/Confessions)	1	10	0	1
Transferred to the HGR			0	0
Total outflows	0		3	88
Patients in isolation (End of Day)	76	0 9 147	19	628
including confirmed cases (NC+AC)	462 201	14	0	215
including suspects	261	133	19	413
Overall occupancy rate	90%	136%	42.2%	89.4%

Mental health activities and psychosocial support

Table 7. Key indicators of mental health and psychosocial support activities as of July 2, 2026

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	2 (40%)	2 (100.0%)	0
Confirmed cases currently being monitored that have received interventions SMSPS	65 (56.5%)	2 (100.0%)	0
New suspected cases that benefited from SMSPS interventions: 12 (63.1%)		6 (100.0%)	0
Suspected cases under follow-up who received interventions: 70 (67.9%) SMSPS		17 (100.0%)	3 (100.0%)
Laboratory results have been announced.	32	13	1
Including positive ones	2	2	0
Number of children separated in daycare who benefited from interventions SMSPS	1 (100.0%)	0	0
Number of children separated in the community who benefited from SMSPS interventions	0	0	9
Number of accompanying persons at the CTE who benefited from interventions SMSPS	57	8	0
Number of PPLs who benefited from SMSPS interventions	14	3	0
Number of household and community members who benefited from SMSPS interventions	15	3	0
Number of EDS supported	14	2	0

4.6 Continuity of care

- 20 midwives and nurses from the Bambu health zone in Ituri were trained on sexual and reproductive health in the context of Ebola.
- A total of **102 PPLs** have been confirmed, including 33 deaths (24.5%) and **36 recoveries** in the province of Ituri.

4.7 Risk communication and community engagement

Ituri: One (1) meeting was organized in the Nizi Health Zone for 46 participants to obtain community commitment to fight against Ebola and their adherence to the commitment and fight against Ebola; 138 social mobilization activities (community dialogues and mass awareness) reaching 262,070 (25,836 men and 236,234 women) on preventive measures of Ebola, community-based surveillance, care, decontamination and EDS in the Health Zones of Mongbwalu, Nyankunde, Nizi, Rwampara, Nia-Nia, Aungba, Bambu, Aru; 279 educational talks and home visits organized by 912 Recos, 9,950 households were reached and 17,564 people reached (6,594 men and 10,972 women) in the Health Zones of Bunia and NiaNia; Briefing of 89 service providers on community-based surveillance, CREC with practical exercises on the organisation of home visits, facilitation of educational talks in their respective environments and the use of tools (community definition of cases, contact registration form, contact tracing form, and home visit form) in the health zones of Mongbwalu and Bunia; ; 19 broadcasts of spots and messages on preventive measures for Ebola Virus Disease (EVD) (community definition of cases, contact registration form, contact tracing form, and home visit form); 18 broadcasts of spots and messages on preventive measures for EVD.

Community feedback: Improve patient care; Ensure handwashing stations are available on street corners; Support churches with the use of additional equipment; Encourage survivors of Ebola Treatment Centers (ETCs) to begin raising awareness themselves in the field.

Limit the number of passengers on public transport; Why do some cases declared recovered relapse some time later? Some patients return home due to overcrowding at the treatment center.

Support for the pillars of the

response : Surveillance: Support for listing confirmed cases.

PCI: Support for the surveillance, PCI, ESD pillars in the Nia-Nia health zone.

North Kivu: Three briefing sessions for taxi drivers including 90 leaders of the Beni motorcyclists, 69 managers of private health structures and 20 members of the CAC on respecting barrier measures, strengthening community-based surveillance and the CREC for the prevention of Ebola virus disease.

4.8. Logistics

Ituri

- Receipt of two Toyota Land Cruiser vehicles, a donation from UNICEF to the DPS Ituri.
- Provision of 26 vehicles and 4 ambulances to the pillars and main health zones affected.
- Work continues on the development of the Elikya CTE, with 14 tents available, a laboratory in construction and electrical installation work.
- Delivery of communication materials to the CREC pillar and participation in the quantification of specific drug and input needs for Ituri, North Kivu, South Kivu, Haut-Uélé and Tshopo.

North Kivu

- Receipt of the 2nd batch of medicines / donation from the WHO
- Delivery of supplies to the Beni Health Zone: 1 Laser Jet cartridge /222 and 38 packages of medicines/donation UG PDSS
- Supply of PCI inputs to the Oicha ZS (50 pieces of Tayvek, i.e., 2 packages of 25 pieces)
- Delivery of 370 packages of medicines/donation from UG PDSS to the Mutwanga Health Zone
- Packaging of PCI inputs, Surveillance and other medical care medications for the M'PA ZS (30 pairs of protective goggles, 2 liters of hydroalcoholic gel, 15 pieces of liquid soap, 4 cartons of 20-piece bar soap, 5 pieces of metal beds and 5 pieces of padded mattresses)
- Monitoring the repair of the vehicle provided by the ICCN currently being repaired in MUTSORA.

4.9. Security

- The security situation remains calm but unpredictable in Bunia and surrounding areas.
- The incident at PK51 in the Nia-Nia Health Zone has disrupted planned activities. Strengthening dialogue with local authorities, community leaders, and security services remains a priority to ensure access to high-risk areas.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

Ituri : Awareness-raising activities and installation of PSEA (Public Service for Agriculture and Forestry) materials reached 231 people, including 180 women and 51 men, in the Governorate, in markets, at the Tséré site, and in several structures.

health in Bunia and Rwampara. Actions included the distribution of advice cards, the display of messages and community awareness-raising.

5. MAIN CHALLENGES, IMPACTS AND REQUIRED ACTIONS

Challenge	Impact	Actions required
No. 1 Reluctance to undergo post-mortem sampling (EDS/swabs) and community resistance	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, and strengthen team security.
2 Insufficient capacity of CTEs and saturation in North Kivu (138.8%)	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTE, deploy additional temporary structures
3. Performance of the monitoring of contacts not optimal (81.3% vs target 95%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs), enhanced supervision
4. Low reporting of alerts in certain affected zones	Undetected transmission, silent spread of the epidemic	Strengthen community surveillance, improve the reporting and rapid investigation of alerts
5 confirmed cases not yet broken down by health zone (17 case)	Major gaps in the reconstruction of transmission chains	Conducting a data reconciliation, Remove any possible duplicates and reclassify. or specify the health zone and status (deceased, recovered, active, etc.). Active verification on the ground.
6 Insufficient supply of MEG and medical supplies	Weakening of overall care	Advocacy and urgent supply of essential medicines
7 Insufficient PCI inputs (PPE, chlorine) and limited training	High risk of nosocomial infections (96 cases) HCW (including 19 deaths, case fatality rate 19.7%)	Urgent supply, strengthening of IPC training
8 Stockpile of unanalyzed samples (backlog) at the laboratory in North Kivu and delayed diagnosis	Delayed confirmation, high lethality (54.2%)	Deploy a laboratory catch-up plan, strengthen diagnostic capabilities
9 Insufficient number of ambulances and isolation facilities (gap of 20 centers)	Transfer delays and isolation increase the risk of transmission	Urgent deployment of additional logistical resources and infrastructure
10. Insufficient coordination and low utilization of COUSP frameworks	Operational inefficiency and delayed decisions	Strengthen coordination mechanisms at all levels, make operational the Provincial COUSP
11. Insufficient resources financial (gap of 20 million USD)	Limitation of the implementation of all pillars of the response	Urgent mobilization of resources, advocacy with partners
12 Insecurity and limited access (CODECO, ADF)	Restrictions on operations, limited access to high-risk areas	Strengthening security coordination and humanitarian access
13 Continuity of essential services (CEHS) compromised	Increase in indirect mortality (not Ebola)	Effective implementation of free healthcare, strengthening of CEHS
14. High population mobility	High risk of rapid geographic expansion	Strengthening surveillance at PoE/PoC sites and cross-border coordination

6. PHOTOS OF FIELD ACTIVITIES



Installation of the diagnostic laboratory in the Komanda health zone



Guided tour of the CTE Beni for young leaders to overcome resistance to the transfer of Ebola patients

**POUR TOUTE INFORMATION
SUPPLÉMENTAIRE, VEUILLEZ CONTACTER**

**Pour l'Institut National de la Santé Publique
(INSP) de la RDC**

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**Avec l'appui technique de l'Organisation Mondiale
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